

1. NAME OF THE MEDICINAL PRODUCT

Ospamox® 250 mg/5 mL Granules for Oral Suspension.

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each 5 mL reconstituted oral suspension contains Amoxicillin 250 mg; Sodium Benzoate 7.1 mg; Aspartame 8.5 mg.

3. PHARMACEUTICAL FORM

Powder for oral suspension.

4. CLINICAL PARTICULARS

4.1 Therapeutic Indications

Ospamox is useful in the treatment of infections caused by amoxicillin-susceptible organisms:

- Acute bacterial sinusitis
- Acute otitis media
- Acute streptococcal tonsillitis and pharyngitis
- Acute exacerbations of chronic bronchitis
- Community acquired pneumonia
- Acute cystitis
- Asymptomatic bacteriuria in pregnancy
- Acute pyelonephritis
- Typhoid and paratyphoid fever
- Dental abscess with spreading cellulitis
- Prosthetic joint infections
- *Helicobacter pylori* eradication
- Lyme disease

Amoxicillin is also indicated for the prophylaxis of endocarditis.

Consideration should be given to official guidance on the appropriate use of antibacterial agents.

4.2 Posology and method of administration

Posology

The dose of amoxicillin that is selected to treat an individual infection should take into account:

- The expected pathogens and their likely susceptibility to antibacterial agents (see section 4.4)
- The severity and the site of the infection
- The age, weight and renal function of the patient; as shown below

The duration of therapy should be determined by the type of infection and the response of the patient, and should generally be as short as possible. Some infections require longer periods of treatment (see section 4.4 regarding prolonged therapy).

Adults and children ≥40 kg

Indication*	Dose*
Acute bacterial sinusitis	250 mg to 500 mg every 8 hours or 750 mg to 1 g every 12 hours
Asymptomatic bacteriuria in pregnancy	
Acute pyelonephritis	For severe infections 750 mg to 1 g every 8 hours
Dental abscess with spreading cellulitis	Acute cystitis may be treated with 3 g twice daily for one day
Acute cystitis	
Acute otitis media	500 mg every 8 hours, 750 mg to 1 g every 12 hours For severe infections 750 mg to 1 g every 8 hours for 10 days
Acute streptococcal tonsillitis and pharyngitis	
Acute exacerbations of chronic bronchitis	
Community acquired pneumonia	500 mg to 1 g every 8 hours
Typhoid and paratyphoid fever	500 mg to 2 g every 8 hours
Prosthetic joint infections	500 mg to 1 g every 8 hours

Prophylaxis of endocarditis	2 g orally, single dose 30 to 60 minutes before procedure
<i>Helicobacter pylori</i> eradication	750 mg to 1 g twice daily in combination with a proton pump inhibitor (e.g. omeprazole, lansoprazole) and another antibiotic (e.g. clarithromycin, metronidazole) for 7 days
Lyme disease (see section 4.4)	Early stage: 500 mg to 1 g every 8 hours up to a maximum of 4 g/day in divided doses for 14 days (10 to 21 days) Late stage (systemic involvement): 500 mg to 2 g every 8 hours up to a maximum of 6 g/day in divided doses for 10 to 30 days
*Consideration should be given to the official treatment guidelines for each indication	

Children <40 kg

Children weighing 40 kg or more should be prescribed the adult dose.

Recommended doses

Indication ⁺	Dose ⁺
Acute bacterial sinusitis	20 to 90 mg/kg/day in divided doses*
Acute otitis media	
Community acquired pneumonia	
Acute cystitis	
Acute pyelonephritis	
Dental abscess with spreading cellulitis	
Acute streptococcal tonsillitis and pharyngitis	40 to 90 mg/kg/day in divided doses*
Typhoid and paratyphoid fever	100 mg/kg/day in three divided doses
Prophylaxis of endocarditis	50 mg/kg orally, single dose 30 to 60 minutes before procedure
Lyme disease (see section 4.4)	Early stage: 25 to 50 mg/kg/day in three divided doses for 10 to 21 days Late stage (systemic involvement): 100 mg/kg/day in three divided doses for 10 to 30 days
+ Consideration should be given to the official treatment guidelines for each indication.	
* Twice daily dosing regimens should only be considered when the dose is in the upper range.	

Elderly

No dose adjustment is considered necessary.

Renal impairment

GFR (ml/min)	Adults and children ≥40 kg	Children <40 kg [#]
greater than 30	no adjustment necessary	no adjustment necessary
10 to 30	maximum 500 mg twice daily	15 mg/kg given twice daily (maximum 500 mg twice daily)
less than 10	maximum 500 mg/day.	15 mg/kg given as a single daily dose (maximum 500 mg)
[#] In the majority of cases, parenteral therapy is preferred.		

In patients receiving haemodialysis

Amoxicillin may be removed from the circulation by haemodialysis.

	Haemodialysis
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Adults and children ≥ 40 kg	15 mg/kg/day given as a single daily dose. Prior to haemodialysis one additional dose of 15 mg/kg should be administered. In order to restore circulating active substance levels, another dose of 15 mg/kg should be administered after haemodialysis.
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In patients receiving peritoneal dialysis

Amoxicillin maximum 500 mg/day.

Hepatic impairment

Dose with caution and monitor hepatic function at regular intervals (see sections 4.4 and 4.8).

Method of administration

Amoxicillin is for oral use.

Absorption of amoxicillin is unimpaired by food.

Powder for oral suspension

For instructions on reconstitution of the medicinal product before administration, see section 6.5.

The preparation is administered orally with a measuring spoon and taken with a glass of water.

4.3 Contraindications

Hypersensitivity to the active substance, to any of the penicillins or to any of the excipients.

History of a severe immediate hypersensitivity reaction (e.g. anaphylaxis) to another beta-lactam agent (e.g. a cephalosporin, carbapenem or monobactam).

4.4 Special warnings and precautions for use

Hypersensitivity reactions

Before initiating therapy with amoxicillin, careful enquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins or other beta-lactam agents (see sections 4.3 and 4.8).

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients on penicillin therapy. These reactions are more likely to occur in individuals with a history of penicillin hypersensitivity and in atopic individuals.

If an allergic reaction occurs, amoxicillin therapy must be discontinued and appropriate alternative therapy instituted.

NPRA Directive

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Ospamox, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Ospamox must be discontinued and appropriate alternative therapy instituted.

Non-susceptible microorganisms

Amoxicillin is not suitable for the treatment of some types of infection unless the pathogen is already documented and known to be susceptible or there is a very high likelihood that the pathogen would be suitable for treatment with amoxicillin (see section 5.1). This particularly applies when considering the treatment of patients with urinary tract infections and severe infections of the ear, nose and throat.

Convulsions

Convulsions may occur in patients with impaired renal function or in those receiving high doses or in patients with predisposing factors (e.g. history of seizures, treated epilepsy or meningeal disorders) (see section 4.8).

Renal impairment

In patients with renal impairment, the dose should be adjusted according to the degree of impairment (see section 4.2).

Skin reactions

The occurrence at the treatment initiation of a feverish generalised erythema associated with pustula may be a symptom of acute generalised exanthemous pustulosis (AGEP, see section 4.8). This reaction requires amoxicillin discontinuation and contra-indicates any subsequent administration.

Amoxicillin should be avoided if infectious mononucleosis is suspected since the occurrence of a morbilliform rash has been associated with this condition following the use of amoxicillin.

Jarisch-Herxheimer reaction

The Jarisch-Herxheimer reaction has been seen following amoxicillin treatment of Lyme disease (see section 4.8). It results directly from the bactericidal activity of amoxicillin on the causative bacteria of Lyme disease, the spirochaete *Borrelia burgdorferi*. Patients should be reassured that this is a common and usually self-limiting consequence of antibiotic treatment of Lyme disease.

Overgrowth of non-susceptible microorganisms

Prolonged use may occasionally result in overgrowth of non-susceptible organisms.

Antibiotic-associated colitis has been reported with nearly all antibacterial agents and may range in severity from mild to life threatening (see section 4.8). Therefore, it is important to consider this diagnosis in patients who present with diarrhoea during, or subsequent to, the administration of any antibiotics. Should antibiotic-associated colitis occur, amoxicillin should immediately be discontinued, a physician consulted and an appropriate therapy initiated. Anti-peristaltic medicinal products are contraindicated in this situation.

Prolonged therapy

Periodic assessment of organ system functions; including renal, hepatic and haematopoietic function is advisable during prolonged therapy. Elevated liver enzymes and changes in blood counts have been reported (see section 4.8).

Anticoagulants

Prolongation of prothrombin time has been reported rarely in patients receiving amoxicillin. Appropriate monitoring should be undertaken when anticoagulants are prescribed concomitantly. Adjustments in the dose of oral anticoagulants may be necessary to maintain the desired level of anticoagulation (see section 4.5 and 4.8).

Crystalluria

In patients with reduced urine output, crystalluria has been observed very rarely, predominantly with parenteral therapy. During the administration of high doses of amoxicillin, it is advisable to maintain adequate fluid intake and urinary output in order to reduce the possibility of amoxicillin crystalluria. In patients with bladder catheters, a regular check of patency should be maintained (see section 4.8 and 4.9).

Interference with diagnostic tests

Elevated serum and urinary levels of amoxicillin are likely to affect certain laboratory tests. Due to the high urinary concentrations of amoxicillin, false positive readings are common with chemical methods.

It is recommended that when testing for the presence of glucose in urine during amoxicillin treatment, enzymatic glucose oxidase methods should be used.

The presence of amoxicillin may distort assay results for oestriol in pregnant women.

Powder for oral suspension

Care should be taken in patients with phenylketonuria as the syrup preparations contain Aspartame.

4.5 Interaction with other medicinal products and other forms of interaction

Probenecid

Concomitant use of probenecid is not recommended. Probenecid decreases the renal tubular secretion of amoxicillin. Concomitant use of probenecid may result in increased and prolonged blood levels of amoxicillin.

Allopurinol

Concurrent administration of allopurinol during treatment with amoxicillin can increase the likelihood of allergic skin reactions.

Tetracyclines

Tetracyclines and other bacteriostatic medicinal products may interfere with the bactericidal effects of amoxicillin.

Oral anticoagulants

Oral anticoagulants and penicillin antibiotics have been widely used in practice without reports of interaction. However, in the literature there are cases of increased international normalised ratio in patients maintained on acenocoumarol or warfarin and prescribed a course of amoxicillin. If co-administration is necessary, the prothrombin time or international normalised ratio should be carefully monitored with the addition or withdrawal of amoxicillin. Moreover, adjustments in the dose of oral anticoagulants may be necessary (see section 4.4 and 4.8).

Methotrexate

Penicillins may reduce the excretion of methotrexate causing a potential increase in toxicity.

4.6 Pregnancy and lactation

Pregnancy

Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. Limited data on the use of amoxicillin during pregnancy in humans do not indicate an increased risk of congenital malformations. Amoxicillin may be used in pregnancy when the potential benefits outweigh the potential risks associated with treatment.

Breastfeeding

Amoxicillin is excreted into breast milk in small quantities with the possible risk of sensitisation. Consequently, diarrhoea and fungus infection of the mucous membranes are possible in the breast-fed infant, so that breast-feeding might have to be discontinued. Amoxicillin should only be used during breast-feeding after benefit/risk assessment by the physician in charge.

Fertility

There are no data on the effects of amoxicillin on fertility in humans. Reproductive studies in animals have shown no effects on fertility.

4.7 Effects on ability to drive and use machines

No studies on the effects on the ability to drive and use machines have been performed. However, undesirable effects may occur (e.g., allergic reactions, dizziness, convulsions), which may influence the ability to drive and use machines (see section 4.8).

4.8 Undesirable effects

The most commonly reported adverse drug reactions (ADRs) are diarrhoea, nausea and skin rash.

The following terminologies have been used in order to classify the occurrence of undesirable effects: Very common, Common, Uncommon, Rare, Very rare, Not known.

Infections and infestations

Very rare: Mucocutaneous candidiasis

Blood and lymphatic system disorders

Very rare: Reversible leucopenia (including severe neutropenia or agranulocytosis), reversible thrombocytopenia and haemolytic anaemia, prolongation of bleeding time and prothrombin time (see section 4.4)

Immune system disorders

Very rare: Severe allergic reactions, including angioneurotic oedema, anaphylaxis, serum sickness and hypersensitivity vasculitis (see section 4.4)

Not known: Jarisch-Herxheimer reaction (see section 4.4)

Nervous system disorders

Very rare: Hyperkinesia, dizziness and convulsions (see section 4.4)
Not known: Aseptic meningitis

Gastrointestinal disorders

Common: Diarrhoea and nausea
Uncommon: Vomiting

Post-marketing data

Very rare: Antibiotic associated colitis (including pseudomembranous colitis and haemorrhagic colitis, see section 4.4), black hairy tongue
[Powder for oral suspension only] Superficial tooth discolouration[#]

Hepatobiliary disorders

Very rare: Hepatitis and cholestatic jaundice, moderate rise in AST and/or ALT

Skin and subcutaneous tissue disorders

Common: Skin rash
Uncommon: Urticaria and pruritus

Post-marketing data

Very rare: Skin reactions such as erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis, bullous and exfoliative dermatitis, acute generalised exanthematous pustulosis (AGEP) (see section 4.4) and drug reaction with eosinophilia and systemic symptoms (DRESS)

Renal and urinary tract disorders

Very rare: Interstitial nephritis, crystalluria (see section 4.4 and 4.9)

[Powder for oral suspension only]

[#] Superficial tooth discolouration has been reported in children. Good oral hygiene may help to prevent tooth discolouration as it can usually be removed by brushing.

4.9 Overdose

Symptoms and signs of overdose

Gastrointestinal symptoms (such as nausea, vomiting and diarrhoea) and disturbance of the fluid and electrolyte balances may be evident. Amoxicillin crystalluria, in some cases leading to renal failure, has been observed. Convulsions may occur in patients with impaired renal function or in those receiving high doses (see sections 4.4 and 4.8).

Treatment of intoxication

Gastrointestinal symptoms may be treated symptomatically, with attention to the water/electrolyte balance.

Amoxicillin can be removed from the circulation by haemodialysis.

5. PHARMACOLOGICAL PROPERTIES

5.1 Pharmacodynamics properties

Pharmacotherapeutic group: Antibacterials for systemic use, beta-lactam antibacterials, penicillins, penicillins with extended spectrum
ATC code: J01CA04

Mechanism of action

Amoxicillin is a semisynthetic penicillin (beta-lactam antibiotic) that inhibits one or more enzymes (often referred to as penicillin-binding proteins, PBPs) in the biosynthetic pathway of bacterial peptidoglycan, which is an integral structural component of the bacterial cell wall. Inhibition of peptidoglycan synthesis leads to weakening of the cell wall, which is usually followed by cell lysis and death.

Amoxicillin is susceptible to degradation by beta-lactamases produced by resistant bacteria and therefore the spectrum of activity of amoxicillin alone does not include organisms which produce these enzymes.

Pharmacokinetic/pharmacodynamic relationship

The time above the minimum inhibitory concentration (T>MIC) is considered to be the major determinant of efficacy for amoxicillin.

Mechanisms of resistance

The main mechanisms of resistance to amoxicillin are:

- Inactivation by bacterial beta-lactamases.
- Alteration of PBPs, which reduce the affinity of the antibacterial agent for the target.

Impermeability of bacteria or efflux pump mechanisms may cause or contribute to bacterial resistance, particularly in Gram-negative bacteria.

Breakpoints

MIC breakpoints for amoxicillin are those of the European Committee on Antimicrobial Susceptibility Testing (EUCAST) version 7.1.

Organism	MIC breakpoint (mg/l)	
	Susceptible ≤	Resistant >
Enterobacteriaceae	8 ¹	8
<i>Staphylococcus</i> spp.	Note ^{2,3}	Note ^{2,3}
<i>Enterococcus</i> spp. ⁴	4	8
Streptococcus groups A, B, C and G	Note ⁵	Note ⁵
<i>Streptococcus pneumoniae</i>	Note ⁶	Note ⁶
Viridans group streptococci	0.5	2
<i>Haemophilus influenzae</i>	2 ⁷	2 ⁷
<i>Moraxella catarrhalis</i>	Note ⁸	Note ⁸
<i>Neisseria gonorrhoeae</i> ⁹	Note ⁹	Note ⁹
<i>Neisseria meningitidis</i>	0.125	1
Gram positive anaerobes except <i>Clostridium difficile</i> ¹⁰	4	8
Gram negative anaerobes ¹⁰	0.5	2
<i>Helicobacter pylori</i>	0.125 ¹¹	0.125 ¹¹
<i>Pasteurella multocida</i>	1	1
<i>Aerococcus sanguinicola</i> and <i>urinae</i>	Note ¹²	Note ¹²
<i>Kingella kingae</i>	0.125 ¹³	0.125 ¹³
PK/PD (non-species related) breakpoints	2	8

1. Wild type enterobacteriaceae are categorised as susceptible to aminopenicillins. Some countries prefer to categorise wild type isolates of *E. coli* and *P. mirabilis* as intermediate. When this is the case, use the MIC breakpoint $S \leq 0.5$ mg/l and the corresponding zone diameter breakpoint $S \geq 50$ mm.
2. Most staphylococci are penicillinase producers, which are resistant to benzylpenicillin, phenoxymethylpenicillin, ampicillin, amoxicillin, piperacillin and ticarcillin. Isolates negative for penicillinase and susceptible to methicillin can be reported susceptible to these agents. Isolates positive for penicillinase and methicillin susceptible are susceptible to beta- lactamase inhibitor combinations and isoxazolympenicillins (oxacillin, cloxacillin, dicloxacillin and flucloxacillin). Methicillin resistant isolates are, with few exceptions, resistant to all beta- lactam agents.
3. Ampicillin susceptible *S. saprophyticus* are mecA-negative and susceptible to ampicillin, amoxicillin and piperacillin (without or with a beta-lactamase inhibitor)
4. Susceptibility to ampicillin, amoxicillin and piperacillin with and without beta-lactamase inhibitor can be inferred from ampicillin.
5. The susceptibility of streptococcus groups A, B, C and G to penicillins is inferred from the benzylpenicillin susceptibility with the exception of phenoxymethylpenicillin and isoxazolympenicillins for streptococcus group B.
6. Breakpoints for penicillins other than benzylpenicillin relate only to non-meningitis isolates. Isolates fully susceptible to benzylpenicillin (MIC ≤ 0.06 mg/l and/or susceptible by oxacillin disk screen) can be reported susceptible to beta-lactam agents for which clinical breakpoints are listed (including those with "Note"). For isolates categorised as intermediate to ampicillin avoid oral treatment with amoxicillin, amoxicillin or amoxicillin-clavulanic acid. Susceptibility inferred from the MIC of ampicillin.
7. Breakpoints are based on intravenous administration. Beta-lactamase positive isolates can be reported resistant to ampicillin, amoxicillin and piperacillin without inhibitors. Tests based on a chromogenic cephalosporin can be used to detect the beta-lactamase.
8. Most *M. catarrhalis* produce beta-lactamase, although beta-lactamase production is slow and may give weak results with *in vitro* tests. Beta-lactamase producers should be reported resistant to penicillins and aminopenicillins without inhibitors.
9. Always test for beta-lactamase. If positive, report resistant to benzylpenicillin, ampicillin and amoxicillin. Tests based on a chromogenic cephalosporin can be used to detect the beta-lactamase. The susceptibility of beta-lactamase negative isolates to ampicillin and amoxicillin can be inferred from benzylpenicillin.
10. Susceptibility to ampicillin, amoxicillin, piperacillin and ticarcillin can be inferred from susceptibility to benzylpenicillin.
11. The breakpoints are based on epidemiological cut-off values (ECOFFs), which distinguish wild-type isolates from those with reduced susceptibility.
12. Infer susceptibility from ampicillin susceptibility.
13. Susceptibility can be inferred from benzylpenicillin susceptibility

The prevalence of resistance may vary geographically and with time for selected species, and local information on resistance is desirable, particularly when treating severe infections. As necessary, expert advice should be sought when the local prevalence of resistance is such that the utility of the agent in at least some types of infections is questionable.

In vitro susceptibility of microorganisms to amoxicillin
Commonly susceptible species
Gram-positive aerobes: <i>Enterococcus faecalis</i> Beta-hemolytic streptococci (Groups A, B, C and G) <i>Listeria monocytogenes</i>
Species for which acquired resistance may be a problem
Gram-negative aerobes: <i>Escherichia coli</i> <i>Haemophilus influenzae</i> <i>Helicobacter pylori</i> <i>Proteus mirabilis</i> <i>Salmonella typhi</i> <i>Salmonella paratyphi</i> <i>Pasteurella multocida</i>
Gram-positive aerobes: Coagulase negative staphylococcus <i>Staphylococcus aureus</i> £ <i>Streptococcus pneumoniae</i>

Viridans group streptococcus
Gram-positive anaerobes: <i>Clostridium</i> spp.
Gram-negative anaerobes: <i>Fusobacterium</i> spp.
Other: <i>Borrelia burgdorferi</i>
Inherently resistant organisms[†]
Gram-positive aerobes: <i>Enterococcus faecium</i> [†]
Gram-negative aerobes: <i>Acinetobacter</i> spp. <i>Enterobacter</i> spp. <i>Klebsiella</i> spp. <i>Pseudomonas</i> spp.
Gram-negative anaerobes: <i>Bacteroides</i> spp. (many strains of <i>Bacteroides fragilis</i> are resistant).
Others: <i>Chlamydia</i> spp. <i>Mycoplasma</i> spp. <i>Legionella</i> spp.
[†] Natural intermediate susceptibility in the absence of acquired mechanism of resistance. [£] Almost all <i>S.aureus</i> are resistant to amoxicillin due to production of penicillinase. In addition, all methicillin-resistant strains are resistant to amoxicillin.

5.2 Pharmacokinetics

Absorption

Amoxicillin fully dissociates in aqueous solution at physiological pH. It is rapidly and well absorbed by the oral route of administration. Following oral administration, amoxicillin is approximately 70% bioavailable. The time to peak plasma concentration (T_{max}) is approximately one hour.

In the range 250 to 3000 mg, the bioavailability is linear in proportion to dose (measured as C_{max} and AUC). The absorption is not influenced by simultaneous food intake.

Haemodialysis can be used for elimination of amoxicillin.

Distribution

About 18% of total plasma amoxicillin is bound to protein and the apparent volume of distribution is around 0.3 to 0.4 l/kg.

Following intravenous administration, amoxicillin has been found in gall bladder, abdominal tissue, skin, fat, muscle tissues, synovial and peritoneal fluids, bile and pus. Amoxicillin does not adequately distribute into the cerebrospinal fluid.

From animal studies there is no evidence for significant tissue retention of drug-derived material. Amoxicillin, like most penicillins, can be detected in breast milk (see section 4.6). Amoxicillin has been shown to cross the placental barrier (see section 4.6).

Biotransformation

Amoxicillin is partly excreted in the urine as the inactive penicilloic acid in quantities equivalent to up to 10 to 25% of the initial dose.

Elimination

The major route of elimination for amoxicillin is via the kidney.

Amoxicillin has a mean elimination half-life of approximately one hour and a mean total clearance of approximately 25 l/hour in healthy subjects. Approximately 60 to 70% of the amoxicillin is excreted unchanged in urine during the first 6 hours after administration of a single 250 mg or 500 mg dose of amoxicillin. Various studies have found the urinary excretion to be 50-85% for amoxicillin over a 24 hour

period.

Concomitant use of probenecid delays amoxicillin excretion (see section 4.5).

Age

The elimination half-life of amoxicillin is similar for children aged around 3 months to 2 years and older children and adults. For very young children (including preterm newborns) in the first week of life the interval of administration should not exceed twice daily administration due to immaturity of the renal pathway of elimination. Because elderly patients are more likely to have decreased renal function, care should be taken in dose selection, and it may be useful to monitor renal function.

Gender

Following oral administration of amoxicillin to healthy males and female subjects, gender has no significant impact on the pharmacokinetics of amoxicillin.

Renal impairment

The total serum clearance of amoxicillin decreases proportionately with decreasing renal function (see sections 4.2 and 4.4).

Hepatic impairment

Hepatically impaired patients should be dosed with caution and hepatic function monitored at regular intervals.

6. PHARMACEUTICAL PARTICULARS

6.1 Incompatibilities

Not applicable.

6.2 Shelf-life

If properly stored, Ospamox granules for oral suspension retain their full potency to the date of expiration shown on the pack.

6.3 Special precautions for storage

Powder for oral suspension: Store below 30°C, protect from light and moisture.

Keep the bottle tightly closed in order to protect from moisture.

Reconstituted suspension: Store in a refrigerator (2°C - 8°C). The reconstituted suspension should be used within 14 days.

6.4 Presentation

Powder for oral suspension

Single packs for 60 mL or 100 mL oral suspensions, hospital packs 40's x 60 mL or 100 mL oral suspensions.

White to slightly yellowish powder.

Not all pack sizes may be marketed.

6.5 Special precautions for disposal and other handling

Powder for oral suspension

After reconstitution, the ready-for-use suspension is white to slightly-yellowish suspension.

This medicinal product should not be used if lumps of powder are visible in the bottle before reconstitution.

After reconstitution the product should not be used if the colour of the reconstituted product is different from the one described before.

After opening of the screw cap, ensure that the bottle cap seal is intact and tightly attached to the bottle rim. Do not use if not intact.

In case of any signs of leakage (e.g. powder outside the bottle), do not use this bottle.

- To reconstitute the suspension, fill the bottle with fresh drinking water approximately 1 cm below the filling mark, close and shake well at once. After the foam has settled, fresh drinking water is slowly added up to the filling mark.
- Shake vigorously again.
- The white to slightly yellowish suspension is now ready for use.

- Shake the bottle well before every withdrawal.

Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

7. PRODUCT REGISTRATION HOLDER

Sandoz Products Malaysia Sdn. Bhd.
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8. DATE OF REVISION OF THE TEXT

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